

Chapter 6 (Revised & Expanded): The Economics of Transformation

Global Budgets, Reference-Based Pricing, and the Financial Architecture of System Change

Sources: Oliver Wyman Act 167 Community Engagement: Recommendations (2024); Act 68 of 2025 (Vermont); GMBB Act 68 RBP/Global Budget Update, February 2026; AHEAD State Agreement (January 2025); Maryland All-Payer Model data (HSCRC, CMS, Commonwealth Fund, Milbank Fund); Vermont RHT Program Application (November 2025); AHS Legislative Presentation (January 2025).

The Economics Pillar: Why Payment Reform Is the Master Variable

Of the six pillars in the HTR framework, the Economics pillar is the one that determines whether the other five actually produce results. You can design perfect care protocols, build excellent data infrastructure, and train a superb clinical workforce — but if the payment system rewards volume over value, every other improvement will be absorbed by the financial logic of fee-for-service. Payment reform is not one element of healthcare transformation. It is the precondition on which all other transformation depends.

This chapter develops the economics of healthcare transformation through three lenses that are analytically inseparable: the mechanics of how hospitals are actually paid today and why that model is failing (with Vermont as the case study), the technical architecture of the alternative payment mechanisms designed to replace it (reference-based pricing and global budgets), and the financial modeling discipline that organizations need to navigate the transition. Vermont's reform journey — from the Act 167 diagnostic through the Act 68 mandate through the AHEAD Model state agreement — is the most complete real-world laboratory for these economic questions currently operating in the United States.

Section 1: The Fee-for-Service Trap — Why the Current System Fails

Fee-for-service payment is the foundational economic structure of American healthcare: providers are paid for each service they deliver, with payment amounts set by negotiated contracts or government rate schedules. The system's economic logic is simple and, from a provider's perspective, entirely rational: more services delivered means more revenue. The problem is that this rationality at the individual transaction level produces irrational outcomes at the system level.

The Volume Incentive Problem

Under fee-for-service, a hospital that prevents a hospitalization loses revenue. A hospital that prevents a readmission loses revenue. A hospital that successfully manages a patient's chronic condition in primary care so they never need the emergency department loses revenue. Every improvement in population health, every success in care coordination, every dollar spent on preventing the expensive acute event — each of these reduces the hospital's income under the fee-for-service model.

This is not a criticism of individual hospital managers or clinicians, who are generally motivated by genuine commitment to patient care. It is an observation about the structural incentive embedded in the payment architecture. When the financial incentive and the clinical incentive point in opposite directions, the financial incentive wins — not always, not in every case, but systematically and at scale. This is why fifteen years of voluntary value-based care programs have moved the needle on payment reform without fundamentally changing the volume-driven behavior of the hospital sector.

"The concept of the global budget is the fundamental piece that flipped the incentives for hospitals in the right direction. It was such a monumental change, a sea change, in the way that hospitals thought about raising revenue."

— Maryland state regulator, quoted in JAMA Network Open study of the Maryland All-Payer Model

The Price Extraction Problem

Fee-for-service payment creates a second structural problem distinct from the volume incentive: it enables hospitals with market power to charge commercial payers prices far above their actual cost of care. In competitive markets, prices are disciplined by competition. In highly concentrated hospital markets — which describe the majority of U.S. hospital markets, and all of Vermont's — there is no effective competitive discipline on commercial prices.

Vermont's price data, documented in the GMCB's February 2026 report and confirmed by independent analyses, is among the most extreme in the country:

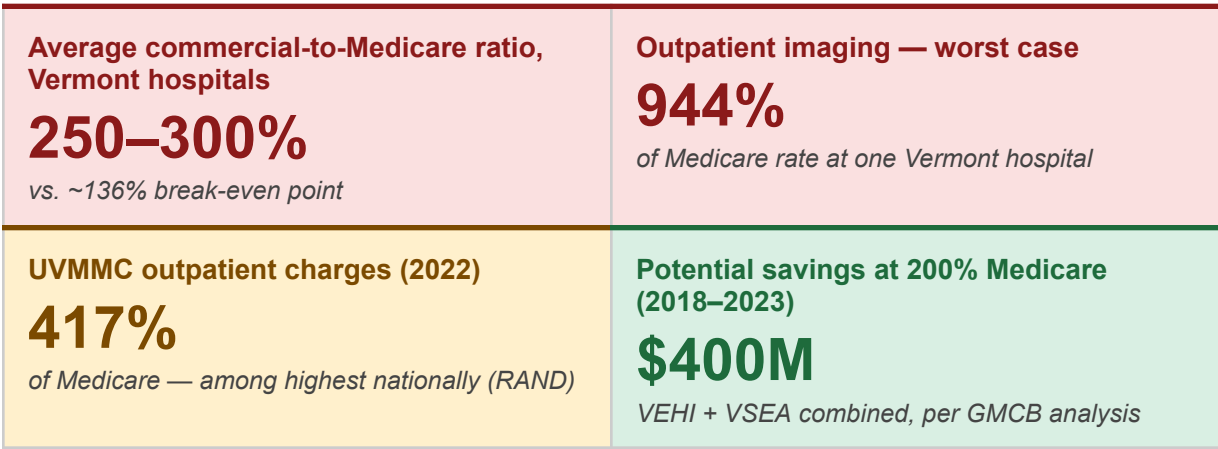


Figure 6.1 — Vermont hospital commercial pricing vs. Medicare and break-even benchmarks. Sources: GMCB Act 68 RBP Update (February 2026); RAND Hospital Price Transparency Study.

The gap between the break-even point (~136% of Medicare) and the actual commercial prices being charged (250–300% on average, with outliers reaching 944%) is not driven by higher quality, better outcomes, or greater complexity. Oliver Wyman's analysis found no relationship between price and quality across Vermont's hospital system. It is driven by market power — the ability of a dominant provider to demand above-cost prices from payers who have no viable alternative for their members.

The consequence for insurance premiums is direct and arithmetically straightforward. When a hospital charges 300% of Medicare for a service instead of 136%, and when commercial insurance covers the majority of that cost, premiums must rise to cover the difference. Vermont's premium trajectory — individual market average monthly premium from \$456 in 2018 to \$948 in 2024, a 108% increase in six years — tracks hospital charge growth with high fidelity. These are not coincidental trends. They are the mathematical output of a price structure that has no regulatory discipline.

Vermont average monthly silver marketplace premium (2018–2024)





Figure 6.2 — Vermont silver marketplace premium trajectory 2018–2024, showing 108% increase over six years. Source: Oliver Wyman Act 167 Report; GMCB analysis.

The Cross-Subsidy Collapse

The fee-for-service model sustains itself through cross-subsidy: commercial payers (who pay above cost) effectively subsidize Medicare and Medicaid patients (who pay below cost). This cross-subsidy is the financial foundation of every U.S. hospital that serves a mixed payer population — which is to say, every hospital.

Vermont's demographic trajectory, documented in detail by Oliver Wyman, is destroying the cross-subsidy model. As the 65+ population expands from 21.7% in 2020 toward 30%+ by 2040, the proportion of patients covered by below-cost Medicare and Medicaid grows. As the working-age population (20-64) declines from 58.8% toward 52%, the pool of commercially insured patients shrinks. The shrinking commercial base must bear a growing cross-subsidy burden — which forces premiums higher — which reduces commercial enrollment — which shrinks the commercial base further. This is not a gradual trend. It is a reinforcing spiral.

Oliver Wyman's financial projections capture where this spiral leads: under the conservative scenario, 13 of Vermont's 14 hospitals report operating losses by 2028, with a cumulative 5-year system deficit of \$700 million to \$2.4 billion depending on expense growth assumptions. The hospital system does not gradually lose efficiency under this trajectory. It collapses.

Section 2: Reference-Based Pricing — Mechanics, Evidence, and Vermont's Design

Reference-based pricing (RBP) is the first operational tool in Vermont's economics reform sequence. It addresses the price extraction problem directly: rather than allowing hospitals and payers to negotiate prices in bilateral, opaque arrangements where hospital market power determines the outcome, RBP establishes a transparent external benchmark — typically a percentage of Medicare reimbursement — as the maximum a hospital may accept.

How RBP Works: The Mechanism

The core RBP mechanism is straightforward: the regulatory authority (in Vermont's case, GMCB) establishes, by rule, the maximum amounts that hospitals shall accept as payment in full for items and services delivered. These maximum amounts are calculated as a percentage of Medicare reimbursement for the same or similar service. The percentage is calibrated to allow hospitals to cover their actual costs (break-even at ~136% of Medicare) while eliminating the excessive markup that drives premium inflation.

Act 68's RBP architecture reflects four design principles that distinguish Vermont's approach from weaker price transparency experiments:

- **Mandatory, not voluntary — every hospital, every commercial payer, by FY2027**
- **Binding ceiling, not advisory guidance — hospitals cannot charge or collect more than the established rate**
- **Transparent benchmark — Medicare rates provide a nationally comparable, publicly available anchor**
- **Premium passthrough monitoring — GMCB and DFR must verify that price reductions translate to lower premiums**

GMCB's February 2026 report introduced one additional design refinement that reflects the Vermont-specific complexity of applying RBP to a highly varied 14-hospital system. Rather than applying a single statewide RBP multiplier, GMCB will consider each hospital's specific community context: payer mix, labor costs, social risk factors, and role in Vermont's system. A critical access hospital serving a predominantly Medicaid and Medicare rural population has different cost economics than a large PPS academic medical center. Vermont's RBP methodology will accommodate these differences while maintaining the essential principle that commercial prices may not exceed a defensible multiple of Medicare.

The Evidence Base: Cross-State Results

Vermont is not designing RBP in a vacuum. Three states have implemented meaningful price reference programs with documented results, providing the evidence base for Vermont's design choices.

State / Program	Design	Scope	Key Results
Oregon (2019)	200% of Medicare in-network; 185% out-of-network. CAH exemptions.	State employee plan (~300,000 covered lives)	\$107.5M saved in first 27 months (Health Affairs). All 24 affected hospitals remained in-network. No documented access harm.
Montana (2016)	Direct contracting at % of Medicare for state employee plan.	State employee plan	\$48M initial savings. Politically fragile without statutory mandate — illustrates Vermont's correct choice to legislate rather than demonstrate.

Washington (2021)	'Cascade Care' public option. 160% of Medicare for participating networks.	Voluntary public option	Expanding to state employee plans. Growing commercial adoption.
Maryland (2014–present)	All-payer rate setting; hospitals receive fixed annual revenue (global budget). Medicare waiver.	All payers, all hospitals statewide	\$1.4B in Medicare hospital savings to date. 7% reduction in hospital admissions. \$800M reduction in hospital spending. Model extended and expanded.
Vermont Act 68 (FY2027)	Mandatory statewide RBP maximum set by GMCB rule. Initially commercial payers; excludes Medicare/Medicaid.	All Vermont hospitals, all commercial payers	Targeting implementation FY27; first negative commercial rate benchmark (-1%) in effect FY26 as transition step.

Figure 6.3 — Cross-state reference-based pricing and rate regulation: design and outcomes. Sources: GMCB Act 68 Update (February 2026); Health Affairs; Commonwealth Fund; CMS.

The Oregon results are the most directly applicable to Vermont's situation because Oregon implemented RBP for a state employee plan while Vermont is implementing it statewide. The critical finding — all 24 affected hospitals remained in-network — directly addresses the most frequently raised concern about RBP: that hospitals will walk away from the regulated rates and leave patients without in-network access. The Oregon evidence is unambiguous on this point: hospitals did not exit the market when rates were set at 200% of Medicare, because 200% of Medicare is still substantially above their actual costs.

What RBP Does — and What It Does Not Do

GMCB's February 2026 report was notably precise about the scope of RBP's impact. Understanding what RBP accomplishes, and what requires complementary mechanisms, is essential for policymakers and healthcare leaders designing transformation strategies.

RBP does:

- Reduce hospital leverage to demand excessive prices from commercial payers
- Narrow unjustified price variation across hospitals for the same service
- Protect patient affordability by establishing binding price ceilings
- Limit monopolistic pricing power in concentrated markets
- Incentivize operational efficiencies over price increases as the margin lever
- Serve as a necessary input for global budget design by establishing the price denominator

RBP does NOT:

- Control total hospital spending — only the price component, not volume
- Guarantee savings pass-through to consumers without explicit monitoring requirements
- Restructure incentives around prevention or population health
- Guarantee hospital efficiency, sustainability, or quality performance
- Solve the cross-subsidy collapse driven by demographic aging

This is why Act 68 sequences RBP first, global budgets second. RBP is a necessary but insufficient tool. Global budgets address what RBP leaves unresolved: total hospital spending, volume incentives, and population health management.

Vermont's Phased Implementation Timeline

GMCB is implementing RBP in two phases, reflecting both the regulatory complexity of establishing a statewide rate-setting methodology and the operational reality that hospitals need runway to adapt their financial models.

Phase	Timeline	Mechanism	Key Actions
Phase I — Differential Price Growth	FY26–FY27 (now underway)	Hospital Budget Guidance	FY27 guidance recommends -1% commercial rate growth benchmark — first negative benchmark in Vermont history. Targets 'outlier services' with highest prices and volumes for initial reductions.
Phase II — Rate Setting (RBP)	FY27–FY28	GMCB Rule-Making	RFP for RBP vendor released December 2025. Discovery and design August 2025–August 2026. Policy and regulatory development January 2026–January 2027. Final RBP methodology released March 2027 for FY28 hospital budget guidance.
Phase III — Global Budget Integration	FY28 and FY30	Statutory Mandate	Non-CAH global hospital budgets by FY28. All-hospital global budgets by FY30. RBP embedded as price component of global budget methodology.

Figure 6.4 — Vermont RBP and global budget implementation phases. Source: GMCB Act 68 Update, February 2026.

The Phase I action — the negative 1% commercial rate benchmark for FY2027 — deserves particular attention because it is already producing results before formal RBP is in place. Combined with FY2026 measures including the Act 55 outpatient drug cap and hospital budget enforcement, GMCB reduced Vermont hospital commercial revenue by \$230.65 million in FY2026: \$104.3M from the Act 55 drug cap, \$94.58M from hospital budget orders, and \$31.76M from budget enforcement. This is the largest single-year price reduction in Vermont hospital history, accomplished through regulatory authority that already existed, signaling what becomes possible when RBP is formally implemented.

Section 3: Hospital Global Budgets — Architecture, History, and Vermont's Mandate

A hospital global budget is a prospectively set annual revenue cap for a hospital, established before the year begins, based on population health needs and past performance rather than on the volume of services delivered during the year. Under a global budget, a hospital receives a fixed total payment regardless of how many patients it sees or how many services it provides. This single design choice — decoupling revenue from volume — fundamentally transforms the hospital's financial incentive structure.

The Economic Logic: Flipping the Incentive

Under fee-for-service, revenue = price × volume. To increase revenue, a hospital must increase either the price per service or the number of services delivered. Under a global budget, revenue is fixed. The only way to improve financial performance is to reduce costs — which means managing population health better, preventing hospitalizations, reducing avoidable readmissions, improving efficiency, and delivering care in less expensive settings.

This is not a theoretical incentive. The Maryland experience, documented over a decade of all-payer global budget operation, demonstrates the behavioral change in concrete terms. As one Maryland state regulator described it: the global budget is the fundamental piece that flips the incentives for hospitals — hospitals had to transition from business strategies that chase volume to ones that focused on population health management. That transition does not happen overnight, but it does happen, and the documented savings are substantial.

The global budget also creates something fee-for-service never can: revenue predictability. Under FFS, a hospital's revenue depends on patient volume, acuity, service mix, and billing accuracy — all variable and partially outside the hospital's control. Under a global budget, the hospital knows its annual revenue before the year begins. This predictability enables the long-term capital planning, workforce investment, and care redesign that hospital transformation requires. For Vermont's financially distressed rural hospitals, revenue predictability is not a secondary benefit — it is a survival mechanism.

Five Design Dimensions Every Global Budget Must Resolve

Global budget design is technically complex, and the design choices are consequential. GMCB's February 2026 report identified five dimensions that any global budget methodology must address, along with the core tradeoff embedded in each. These dimensions apply equally to Vermont's GMCB-designed commercial budgets, the AHEAD Model Medicare budgets, and the Medicaid global budget already in operation.

Design dimension	The choice	Key tradeoff	Vermont's current direction
Scope	Which services are inside vs. outside the budget? (Capital, high-cost drugs, outpatient, physician services?)	Broader scope = more comprehensive cost control. Narrower scope = simpler to administer but leaves cost-shifting opportunities.	TBD through rulemaking. Maryland excludes physician office services and outpatient drugs. Vermont likely to follow similar structure initially.
Population adjustment	How does the budget account for patient demographics, complexity, and socioeconomic factors?	No adjustment rewards hospitals serving healthier populations. Over-adjustment reduces cost-control incentives.	GMCB considering payer mix, labor costs, social risk factors for each hospital. Critical for Vermont's CAHs vs. UVMMC.

Volume treatment	Does the budget adjust if patient volume changes significantly?	Fixed budget = maximum cost-control incentive. Volume-adjusted budget = lower risk for hospitals, less population health incentive.	Maryland preserved revenue initially when volume decreased to ease transition. Vermont likely to adopt similar glide path.
Quality linkage	How are quality metrics integrated — penalty, incentive, or both?	Too small a linkage = no behavioral change. Too large = financial instability for hospitals still improving.	Act 68 requires standardized accountability metrics. GMCB quality programs (readmissions, HACs) will be embedded.
Flexibility mechanisms	Provisions for mid-year adjustments, carryover of savings, risk corridors?	More flexibility = lower hospital risk, easier adoption. Less flexibility = stronger cost-control signal.	GMCB authorized to identify conditions requiring modification or termination of RBP or budget parameters.

Figure 6.5 — Five dimensions of global budget design with Vermont's current direction. Source: GMCB Act 68 Update (February 2026); Act 68 of 2025.

Vermont's Global Budget History: Four Attempts, One Lesson

Vermont has been attempting to implement hospital global budgets for over a decade. The history of those attempts is essential context for understanding why Act 68's mandatory approach represents a fundamental departure from everything that preceded it, and why the departure was necessary.

Approach	Year(s)	Structure	Outcome and lesson
GMCB Net Patient Revenue Cap	2012–present	Regulatory cap on net patient revenue each hospital may receive	Partial constraint. Controlled revenue growth within individual hospital budgets but did not align incentives across payers or address volume gaming.
Rutland Regional Medical Center Pilot	2014 (proposed)	Hospital payments based on historical revenue plus inflation, adjusted for demographics	Never implemented. Hospital opposition and implementation complexity prevented launch. Showed that hospital buy-in is necessary but not sufficient without statutory mandate.
Vermont All-Payer ACO Model	2017–2025	Voluntary commercial payer/provider participation in hospital global payments aligned across Medicare, Medicaid, and commercial based on historical spend	Voluntary participation meant highest-cost commercial payers opted out. OneCare Vermont could not compel participation. Model produced modest results but never achieved the all-payer alignment that makes global budgets effective. Expired end of 2025.
Act 167 / Act 68 Mandate	2022–present	GMCB directed to design and implement mandatory global budgets: non-CAH hospitals by FY2028, all hospitals by FY2030	In progress. Removes the voluntary participation flaw that undermined all prior attempts.

Figure 6.6 — Vermont global budget history: four attempts and the central lesson. Sources: GMCB Act 68 Update (February 2026); Oliver Wyman Act 167 Report.

The central lesson of this history is unambiguous: voluntary models fail because the actors who benefit most from the status quo — the highest-priced hospitals and the commercial payers with the most leverage — can and do opt out. A global budget that is optional for the highest-price hospital in a market is not a global budget; it is a demonstration project. Act 68's statutory mandate eliminates the opt-out. This is the design difference that matters.

"If the model is not adequately regulated and sufficiently resourced, it will fail dramatically and harm patients and harm our health system."
— Owen Foster, Chair of the Vermont Green Mountain Care Board, on signing the AHEAD State Agreement, January 2025

Section 4: Maryland — A Decade of Global Budget Evidence

Maryland is the only U.S. state to have operated a mandatory all-payer hospital rate setting system with global budgets for more than a decade. Its experience is the closest available empirical evidence for what Vermont is attempting, and it deserves detailed examination — both for what it demonstrates is possible and for the limitations the evidence also reveals.

The Maryland Architecture

Maryland's Health Services Cost Review Commission (HSCRC) was established in 1972 with statutory authority to set hospital rates for all commercial payers — a unique regulatory authority that no other state possesses. The HSCRC received a Medicare demonstration waiver in 1977, enabling all-payer rate setting to include Medicare. The current Global Budget Revenue (GBR) methodology, implemented through a renegotiated CMS waiver in 2014, fixes each hospital's total annual revenue before the year begins, regardless of how many patients are served or services provided. Prices for individual services adjust dynamically during the year to meet the fixed revenue target.

The model's accountability structure has three components beyond the global budget itself: a Medicare Performance Adjustment (1% of Medicare revenues) tied to total cost of care outcomes; Quality Based Reimbursement (2% of all-payer revenue) linked to patient experience, safety, and outcomes; and incentives for reducing hospital-acquired conditions, potentially avoidable utilization, and readmissions. The budget constraint is non-negotiable; the quality components are calibrated to reward the population health management behavior the global budget is designed to incentivize.

The Results

The evidence from Maryland's global budget model is substantial and consistent across multiple independent evaluations:

Medicare hospital savings to date \$1.4B vs. projected national trend (HSCRC)	Reduction in hospital admissions 7% Medicare patients; avoidable admissions down >6%
Reduction in hospital spending \$800M Medicare Part A and B combined (CMS evaluation)	All-payer per capita growth cap 3.58% committed growth rate under Maryland model

Figure 6.7 — Maryland All-Payer Global Budget Model results summary. Sources: HSCRC; CMS; Commonwealth Fund; Milbank Memorial Fund.

Maryland also reduced hospital spending for commercial beneficiaries, cut ED visits and inpatient admissions across all payer types, and significantly improved outcomes for dual-eligible (Medicare/Medicaid) patients — the highest-cost, highest-complexity population segment that Vermont also needs to serve better. The 14-day follow-up visit rate and 30-day unplanned readmission rate both improved for non-teaching hospitals.

The Limitations — What Maryland Teaches About What Global Budgets Cannot Do Alone

The Maryland evidence is not uniformly positive, and intellectual honesty requires presenting the limitations alongside the achievements. The Milbank Fund's evaluation found mixed results on quality metrics and no improvement in overall population health measures like smoking rates or obesity prevalence. The reason is analytically important: the global budget creates a powerful incentive to reduce hospital utilization, but it does not automatically create the community infrastructure needed to address the health determinants that drive that utilization.

When Maryland reduced avoidable hospital admissions, the reduction was concentrated among healthier patients — people who, with adequate primary care and social support, would not have needed hospitalization. The sickest patients continued arriving at emergency departments, and hospitals actually spent more time treating them. The lesson is that a global budget without commensurate investment in primary care, behavioral health, housing, and transportation does not improve population health — it just reduces hospital revenue while leaving the underlying drivers of high utilization intact.

Vermont has heard this lesson clearly. The AHEAD State Agreement explicitly links hospital global budgets with the Equity, Access, and Statewide Transformation (EAST) Fund — providing up to \$150 million in additional Medicare funds annually starting in 2027 to invest in primary care, mental health, home health, and long-term care services. The Rural Health Transformation Program award of \$195 million provides capital for workforce, telehealth, and community infrastructure. Vermont's global budget design is, from the beginning, paired with the community investment that Maryland added only after its first years of operation.

Section 5: The AHEAD Model — Medicare's Entry into Vermont's Reform

The AHEAD (Achieving Healthcare Efficiency through Accountable Design) Model represents the federal government's partnership with states committed to genuine all-payer total cost of care transformation. Vermont signed its AHEAD State Agreement on January 17, 2025, committing to participate in Cohort 2 beginning January 1, 2027. The agreement was a 3-1 vote by GMCB, with the dissenting member citing concerns about implementation complexity — a vote that reflects the genuine difficulty and ambition of what Vermont has committed to.

What AHEAD Actually Is

AHEAD is a voluntary state total cost of care model that aims to drive state-level healthcare transformation and multi-payer alignment. The model has three primary components that operate in concert:

- **Hospital Global Budgets — participating hospitals receive prospective, biweekly Medicare FFS payments instead of traditional claims-based payments, providing revenue predictability while creating population health incentives**
- **Primary Care AHEAD — voluntary program for primary care practices to receive enhanced prospective, risk-adjusted Medicare payments for coordinated care, with four payment pathway options including fully prospective models**
- **State accountability — Vermont commits to controlling total cost of care growth, increasing primary care investment as a percentage of total spending, and improving population health outcomes and equity metrics**

The AHEAD Financial Package for Vermont

The AHEAD State Agreement provides Vermont with a substantial financial resource package alongside its accountability commitments:

- Up to \$150 million in additional Medicare funds annually starting in 2027, with inflation adjustments each year
- \$1.2 million in federal transformation grants beginning Cooperative Agreement Year 3 (2027) for health system transformation
- Equity, Access, and Statewide Transformation (EAST) Fund for investment in primary care, behavioral health, home health, and long-term care — the community infrastructure layer that makes global budgets work
- Technical assistance from CMS on global budget design methodology, risk adjustment, and performance monitoring
- Nine-year model duration (through December 31, 2035) providing multi-year planning horizon for hospital transformation

The \$150M annual figure is significant context: Vermont's total Medicare and Medicaid spending exceeds \$3 billion annually. The AHEAD funds represent approximately 5% of that total — meaningful but not transformative in isolation. The value is in what the funds are explicitly directed toward: the community infrastructure and primary care investment that turns a hospital cost-control model into a population health model.

The AHEAD-Act 68 Integration: Why Both Are Required

A critical analytical point that is easily missed: the AHEAD Model and Act 68 are not alternatives or substitutes. They are complementary layers of the same reform architecture, operating on different payer populations and at different speeds, but designed to converge on the same outcome: a Vermont hospital system operating under aligned all-payer global budgets.

AHEAD governs Medicare FFS hospital payments — approximately 23% of Vermont's insured population and a higher share of hospital revenue given Medicare's age-driven

intensity. Act 68 governs commercial hospital prices and budgets — approximately 52% of Vermont's insured population and the majority of hospital revenue. Vermont's Medicaid global budget, already operational under the Global Commitment to Health demonstration and aligned with AHEAD, covers approximately 19% of the population. Together, these three layers create the all-payer alignment that makes a global budget system effective.

Payer layer	% of VT insured pop.	Payment mechanism	Effective date	Governing authority
Medicare FFS	~23%	AHEAD Hospital Global Budgets; biweekly prospective payments	January 2027	CMS / AHEAD State Agreement
Medicaid	~19%	Vermont Global Commitment to Health; Medicaid global budget aligned with AHEAD	In operation; aligned with AHEAD 2027	DVHA / CMS Demonstration Waiver
Commercial (incl. self-insured)	~52%	Reference-Based Pricing (FY2027) → Global Hospital Budgets (FY2028+)	RBP FY2027; budgets FY2028	GMCB / Act 68
Medicare Advantage	~31% of Medicare-eligible	Outside AHEAD; traditional managed care contracts	Not currently in scope	CMS MA contracts
Uninsured	~3%	Charity care; DSH payments	Ongoing	GMCB / DVHA

Figure 6.8 — Vermont all-payer architecture under AHEAD + Act 68 combined. Sources: Vermont RHT Program Application (November 2025); AHS AHEAD fact sheet; Act 68.

The Medicare Advantage gap is worth noting. AHEAD applies to Medicare fee-for-service beneficiaries; approximately 31% of Vermont's Medicare-eligible population is in Medicare Advantage plans, which are managed care contracts between CMS and private insurers. These plans operate outside the AHEAD global budget framework. As MA enrollment continues growing nationally, this gap represents an increasing share of the Medicare population that is not subject to Vermont's hospital cost-containment regime — a design limitation that Vermont policymakers are aware of and that will require ongoing attention.

Section 6: Hospital Financial Modeling — Reading the Numbers That Matter

For healthcare leaders navigating the transition to global budgets and reference-based pricing, financial modeling is not an academic exercise. It is the analytical foundation for every strategic decision: which service lines to maintain or discontinue, how much capital investment is prudent, what staffing model is financially sustainable, when and how aggressively to pursue shared services. This section develops the core financial modeling framework for Vermont's hospital sector, drawing on Oliver Wyman's projections and the GMCB's hospital financial data.

The Oliver Wyman Projection Scenarios

Oliver Wyman built two financial scenarios for Vermont's hospital sector through 2028, representing the range between optimistic and realistic assumptions about expense growth. Both scenarios assume the same revenue growth trajectory (3.5% annual non-340B revenue growth); they differ in their expense growth assumptions, reflecting uncertainty about labor market conditions and drug costs.

Scenario	Assumptions and 5-year system deficit
Conservative: 3.5% revenue / 5% expense growth	Non-340B revenue: +3.5% annually. All operating expenses: +5% annually from 2023 baseline.5-year system deficit (2024-2028):• \$700M vs. break-even• \$1.4B vs. 3% operating margin target• FY2023 budget overage of \$106M suggests starting point may be worse than modeled
Realistic: 3.5% revenue / 7-8% expense growth	Non-340B revenue: +3.5% annually. Non-physician labor: +10% annually. Physician fees: +5% annually. Other operating expenses: +7% annually. 340B payments: +3% (inflation only).5-year system deficit (2024-2028):• \$2.4B vs. break-even (\$3,700 per Vermont resident)• \$3.1B vs. 3% operating margin (\$4,800 per Vermont resident)• Vermont hospitals requested \$285M in FY25 budget increases — a data point supporting the realistic scenario

Figure 6.9 — Oliver Wyman Vermont hospital financial projection scenarios. Source: Oliver Wyman Act 167 Community Engagement: Recommendations (2024).

The UVM Health Network requires separate attention because it dominates Vermont's hospital economics. Under the conservative scenario, UVM network hospitals face a \$312M cumulative 5-year deficit against break-even — more than 44% of the total statewide deficit. Under the realistic scenario, this grows to \$1.5B — again roughly 62% of the statewide total. The concentration of Vermont's hospital financial problem in the largest network is analytically important: system transformation strategies that do not address UVMMC's cost structure and overhead will fail to materially improve statewide affordability. Oliver Wyman specifically called out UVMMC's administrative overhead and the proportion of providers engaged in non-patient-care activities as areas requiring examination.

Reading the Metrics That Actually Matter Under Global Budgets

The transition from fee-for-service to global budgets fundamentally changes which financial metrics matter most. Healthcare finance professionals accustomed to FFS environments need to reorient around a different set of indicators.

In a fee-for-service environment, watch:	In a global budget environment, watch:
Net patient revenue growth	Per-member-per-month cost trend vs. global budget cap

Volume growth (admissions, procedures, visits)	Potentially avoidable utilization rate — every avoidable admission is a cost, not revenue
Case mix index — higher acuity = more revenue	Population health outcomes — better outcomes reduce costs
Market share — capturing more patients = more revenue	ED diversion rate — keeping patients out of the ED saves money
Payer mix — more commercial = better margins	Readmission rate — every readmission is a cost without additional revenue
Discharge volume	Days to discharge — length of stay is a cost driver, not a revenue driver
Revenue cycle efficiency — maximize what each claim pays	Total cost of care per attributed member — efficiency across the whole continuum
Operating margin per inpatient day	Cost per quality-adjusted outcome — the fundamental global budget metric

Figure 6.10 — Financial metrics that matter: FFS vs. global budget environment. Compiled from GMCB guidance, Maryland HSCRC methodology, and Act 68 accountability framework.

This metric shift is not cosmetic. It requires fundamental changes to hospital management information systems, internal reporting, board dashboards, and management incentive structures. A hospital whose CFO is still optimizing revenue per discharge when revenue is fixed will not successfully navigate the global budget transition. The CFO in a global budget environment needs to become an expert in population health economics — tracking the care patterns and community conditions that drive utilization, and investing in interventions that reduce avoidable costs.

The Savings Roadmap: Where Vermont's \$400M+ Can Come From

Oliver Wyman's analysis identified the specific sources of direct and indirect savings from Vermont's transformation. Understanding this roadmap is essential for any organization planning its role in Vermont's reformed system.

Sources of direct savings from Vermont hospital transformation (5-year total, Oliver Wyman estimate)

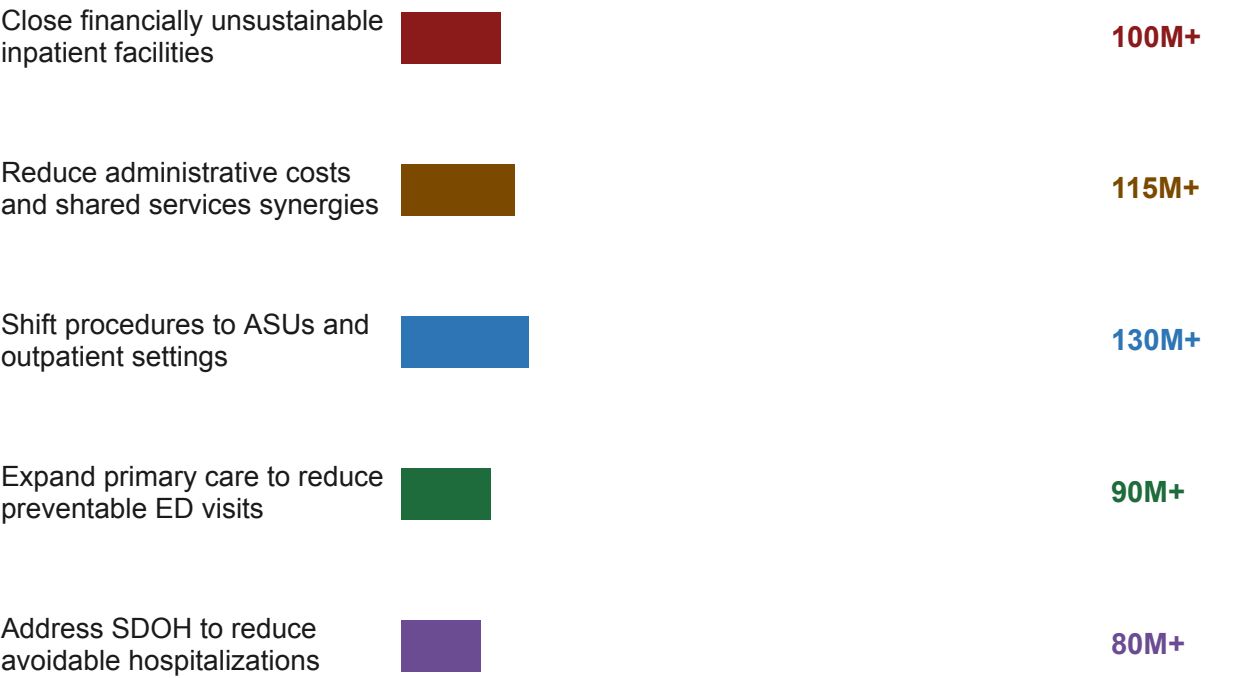


Figure 6.11 — Estimated sources of direct and indirect savings from Vermont hospital transformation. Figures are illustrative ranges derived from Oliver Wyman's >\$400M total estimate. Source: Oliver Wyman Act 167 Report (2024).

The largest single source of indirect savings — shifting procedures from hospital to ambulatory surgical and outpatient settings — reflects the third Oliver Wyman imperative: move all care possible out of hospitals. Low-risk surgical procedures performed in a hospital setting cost two to three times what the same procedure costs in a free-standing ambulatory surgical unit. Vermont currently has only two ambulatory surgical centers (both in Burlington), a significant gap given the breadth of the inpatient surgical capacity Oliver Wyman recommends redirecting. The RHT Program and Act 68 transformation grants are explicitly designed to fund the development of non-hospital care capacity that makes this shift possible.

Section 7: APM Transition Strategy for Vermont Healthcare Organizations

For health systems, payer organizations, and community providers operating in Vermont, the payment reform transformation is not an abstract policy development — it is an operational reality requiring specific preparation. This section provides the strategic and financial planning framework for organizations navigating the transition.

The Three-Phase Transition Model

Vermont's payment reform trajectory follows a predictable three-phase model that organizations can use for planning purposes:

Phase	Timeline	Payment environment	Strategic priorities
Preparation	Now through FY2026	FFS with increasing budget pressure; -1% commercial rate benchmark; Act 55 drug cap in effect	Data infrastructure: build PMPM tracking, attribution, and potentially avoidable utilization dashboards. Workforce: begin training clinical and financial leadership in population health economics. Contracts: audit current payer contracts for RBP transition clauses.
Transition	FY2027	RBP maximum rates in effect for commercial payers; AHEAD Hospital Global Budgets begin January 2027 for Medicare FFS	Revenue modeling: model financial impact of RBP rates on each major service line. Service line rationalization: align with Oliver Wyman COE recommendations. Primary care investment: increase PC capacity ahead of AHEAD's primary care investment requirements.
Operation	FY2028 and beyond	Global hospital budgets for non-CAH hospitals; AHEAD in full operation; Statewide Strategic Plan delivered December 2028	Population health management: from awareness to execution. Quality accountability: GMCB metrics integrated into operational management. Community investment: SDOH programs, care coordination, housing partnerships.

Figure 6.12 — Three-phase APM transition model for Vermont healthcare organizations.

The VBC Readiness Assessment in a Global Budget Context

The HTR Value-Based Care Transformation Readiness Assessment (30 dimensions across six domains) applies directly to Vermont organizations preparing for global budget participation, but three domains deserve elevated attention given the specific demands of the Vermont environment:

- **Data and Technology** — particularly attribution list generation and total cost of care measurement. A hospital that cannot accurately attribute its patient population cannot manage a global budget. Vermont's VHCURES all-payer claims database is the foundational data infrastructure, but many hospitals lack the internal analytics capability to use it effectively for population health management. This gap must be closed before FY2027.
- **Care Delivery Capability** — particularly community health worker deployment and post-acute care network relationships. The Maryland experience demonstrated that global budgets without community care infrastructure simply reduce hospital volume without improving population health. Vermont hospitals must build these relationships proactively.
- **Network and Partnerships** — particularly community partner SDOH referral networks. The Oliver Wyman systems map showed that housing, transportation, and behavioral health gaps drive a significant proportion of avoidable hospital utilization. Hospitals that lack functioning SDOH referral pathways will face cost pressure under global budgets that they cannot address through clinical means alone.

Key Concepts Introduced or Expanded in This Chapter

Term	Definition
Global Budget Revenue (GBR)	Maryland's term for a prospectively fixed annual hospital revenue cap, adjusted for quality performance and population health metrics. The model Vermont's GMCB-designed commercial global budgets will most closely follow.
All-payer alignment	The condition in which Medicare, Medicaid, and commercial payers all operate under the same or compatible payment methodology for hospital services. Essential for global budget effectiveness; Vermont's AHEAD + Act 68 combination is designed to achieve this by FY2028-2030.
Balloon squeeze problem	The dynamic in which controlling hospital prices without controlling volume allows hospitals to compensate through increased service delivery. Reference-based pricing addresses price; global budgets address both price and volume.
Cross-subsidy	The mechanism by which above-cost commercial payments subsidize below-cost Medicare and Medicaid payments. Vermont's demographic shift is making this mechanism financially unsustainable as the commercial base shrinks relative to the government-payer population.
Potentially avoidable utilization (PAU)	Hospital admissions, ED visits, and readmissions that could have been prevented through effective primary care, care coordination, or SDOH intervention. A key performance metric under global budgets; reducing PAU saves money in a capped-revenue environment.
EAST Fund	Equity, Access, and Statewide Transformation Fund — the component of Vermont's AHEAD State Agreement providing up to \$150M annually in additional Medicare funds for investment in primary care, behavioral health, home health, and long-term care.
AHEAD (Achieving Healthcare Efficiency through Accountable Design)	CMMI's voluntary state total cost of care model, with 6 participating states. Vermont participates in Cohort 2, beginning January 2027. Provides Medicare FFS hospital global budgets, enhanced primary care payments, and accountability for total cost of care growth.
Revenue predictability	A primary benefit of global budgets for hospital financial management: knowing total annual revenue before the year begins enables capital planning, workforce investment, and strategic redesign that FFS revenue uncertainty prevents.
PMPM (per member per month)	The standard unit of measurement in population-based payment models. Total spending (or cost) divided by the number of enrolled members divided by the number of months. The fundamental metric for tracking performance under global budgets and capitation arrangements.

Sources: Oliver Wyman Healthcare and Life Sciences Group, *Act 167 Community Engagement: Recommendations* (August 2024, revised October 2024); Act 68 of 2025 (Vermont); Green Mountain Care Board, *Act 68 Update on Hospital Reference-Based Pricing and Global Budgets* (February 17, 2026); Vermont AHEAD State Agreement (January 17, 2025); CMS, *AHEAD Model overview*; Maryland HSCRC, *Global Budget Revenue methodology and annual reports*; Commonwealth Fund, *Hospital Global Budgeting: Lessons from Maryland and Selected Nations* (June 2024); Milbank Memorial Fund, *Uniquely Similar: New Results from Maryland's All-Payer Model* (2020); Vermont Agency of Human Services, *Rural Health Transformation Program Application* (November 2025).